Quality & Patient Safety Brief, 2026 Q3

Lakeshore Community Hospital | data as of Oct 13, 2026

Measure	2026 Q3	Target	vs prior	Status
Falls per 1,000 patient days	2.0	3	better	**Green**
Falls with injury per 1,000 patient days	1.0	1	better	**Amber**
Medication events per 1,000 patient days	3.8	4.5	better	**Green**
CLABSI per 1,000 central-line days	2.1	1	worse	**Red**
CAUTI per 1,000 catheter days	1.5	1.3	worse	**Red**
30-day all-cause readmission rate (%)	11.9	11	worse	**Amber**
30-day readmission, heart failure (%)	22.3	20	worse	**Red**
Inpatient mortality, observed / expected	0.91	1	better	**Green**
SEP-1-style sepsis bundle compliance (%)	69.3	70	better	**Amber**
AMI aspirin within 24h of arrival (%)	94.7	96	worse	**Amber**
HF with LVSD: ACEI/ARB/ARNI at discharge (%)	97.2	90	better	**Green**
VTE prophylaxis by end of admission day 1 (%)	92.9	92	better	**Green**
ED median arrival-to-departure, discharged (min)	160.3	165	worse	**Green**
ED median admit-decision-to-departure (min)	126.0	150	better	**Green**
ED left without being seen (%)	2.0	2.5	worse	**Green**
Safety events reported within 24h (%)	80.7	80	better	**Green**
Safety events closed within 30 days (%)	100.0	85	better	**Green**
Abstraction cases completed by due date (%)	88.6	95	better	**Amber**

What needs attention

Lakeshore Community Hospital | data as of Oct 13, 2026

[High] Fall events on MSW well above that unit's own baseline

2026-03 to 2026-05: 31 events vs 9.1 expected (12.5 vs 3.7 per 1,000 patient days; 3.4x; Poisson p=1e-08).

Suggested action: Review the event narratives for the window, check for a process or equipment change at that time, and report back to the committee with an action plan.

[High] Sepsis bundle compliance dipped well below baseline

Lowest month 2026-01: 45% vs a 67% baseline. In 2025-12 to 2026-02 the weakest element was repeat lactate within 6h (66% vs 86% at baseline).

Suggested action: Check whether order sets or documentation changed in that window; re-educate on the failing element.

[Medium] Medication event rate above the upper control limit

2026-01: 9.4 per 1,000 patient days vs centre line 4.2 (UCL 7.5).

Suggested action: Compare with the timing of system changes (e.g., EHR updates) and sample events for ordering/administration steps.

[Medium] VTE prophylaxis gap: MSW Weekend

MSW Weekend: 79.3% (n=725) vs 92.9% for all other unit/day combinations (chi-square p=1e-38).

Suggested action: Review weekend admission order-set use and pharmacy verification on that unit.

[Medium] 3 measure(s) are Red in the latest quarter

CLABSI per 1,000 central-line days 2.1 (target 1); CAUTI per 1,000 catheter days 1.5 (target 1.3); 30-day readmission, heart failure (%) 22.3 (target 20). Small denominators make single-quarter infection rates volatile; interpret with the control charts.

Suggested action: Review the red measures with the owning department and confirm whether each is a real change or normal variation.

[Medium] Heart-failure 30-day readmissions peaked

2026 Q1: 30.2% (target 20%). Latest quarter 22.3%.

Suggested action: Review post-discharge follow-up scheduling and discharge education for HF patients discharged in the peak quarter.

[Medium] ED throughput signals during the winter surge

Out-of-control points in 2025-10 to 2026-03. ED median admit-decision-to-departure (min): peak 197.3; ED median arrival-to-departure, discharged (min): peak 186.7; ED left without being seen (%): peak 4.0.

Suggested action: Review surge staffing and inpatient bed-flow plans before next winter.

[Medium] Abstraction backlog

On-time completion fell to 38% (centre line 90%) for cases selected 2026-02 to 2026-04.

Suggested action: Rebalance case assignment across abstractors; protect time before quarterly submission deadlines.

[Medium] Public-reporting submission submitted late: SEP1 2025Q4

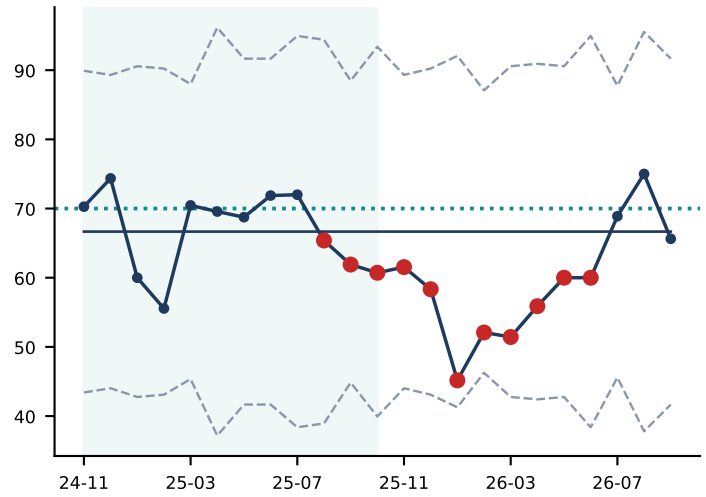
Due 2026-04-30; submitted 2026-05-03.

Suggested action: Add a 30-day reminder to the abstraction calendar and name a backup submitter.

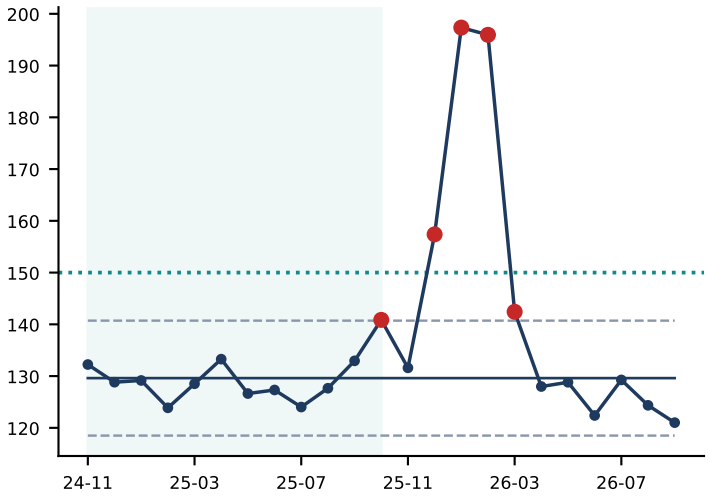
Control charts

Baseline = first 12 months (shaded). Red = signal. Dotted = target.

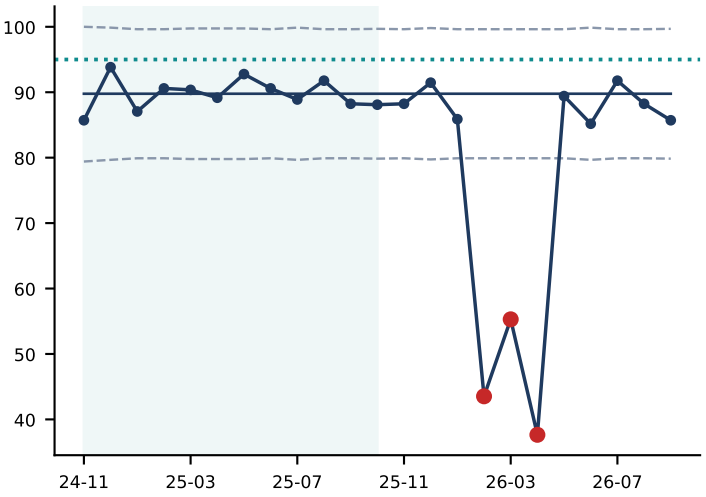
SEP-1-style sepsis bundle compliance (%)



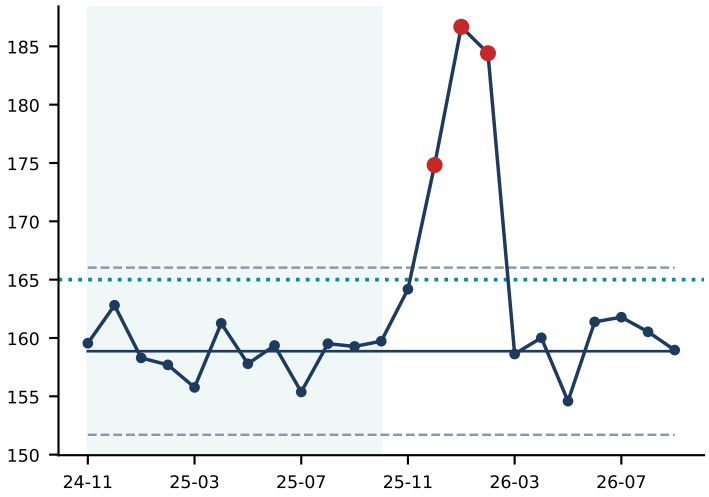
ED median admit-decision-to-departure (min)



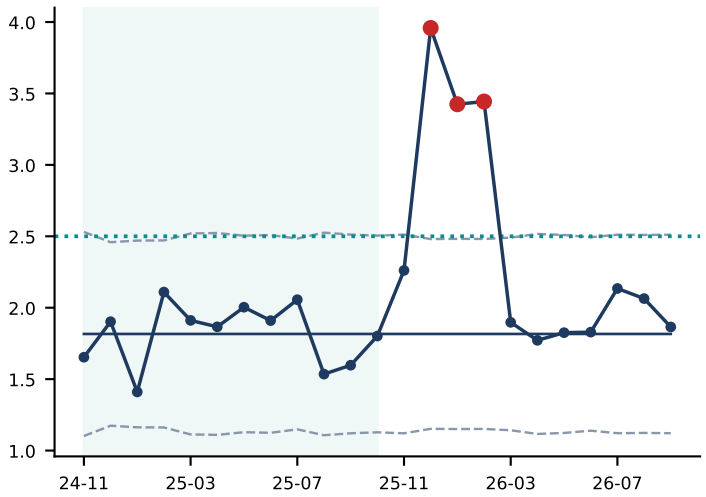
Abstraction cases completed by due date (%)



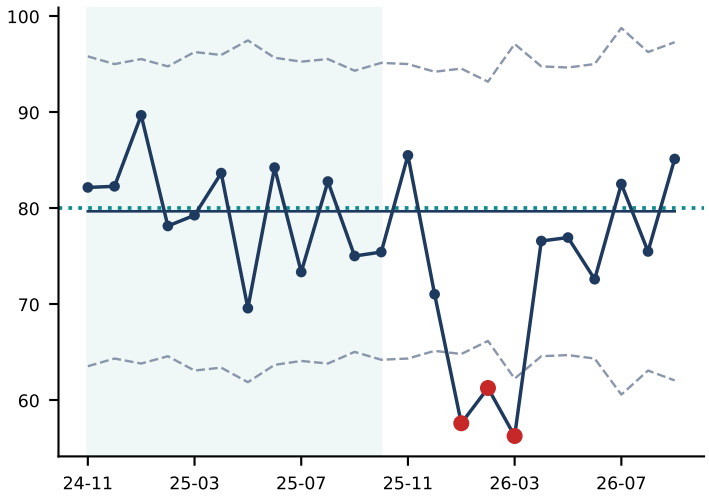
ED median arrival-to-departure, discharged (min)



ED left without being seen (%)



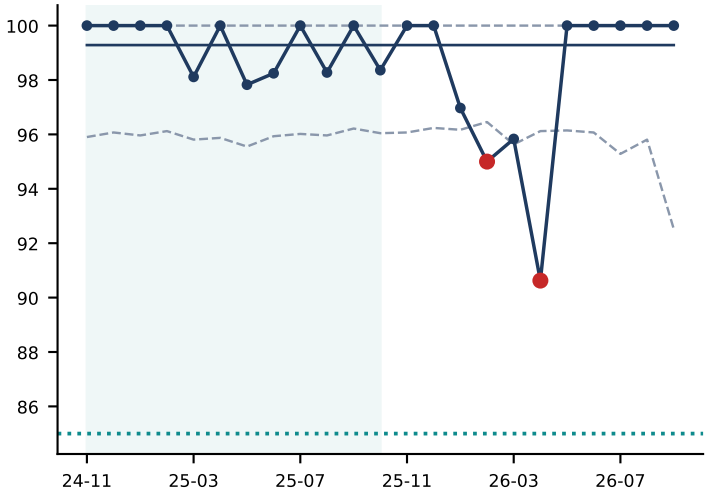
Safety events reported within 24h (%)



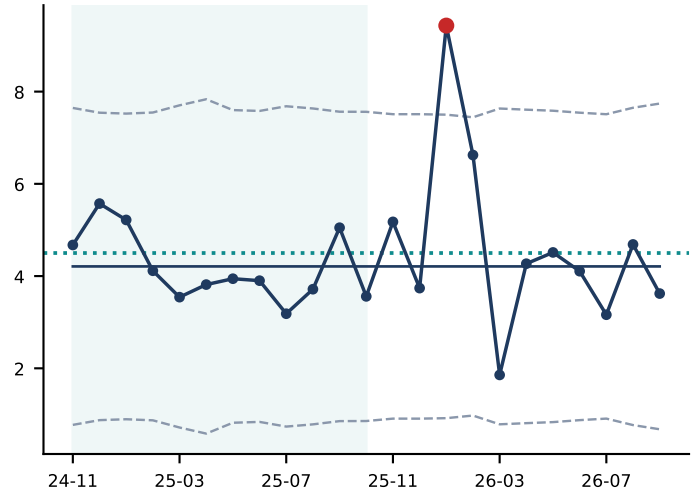
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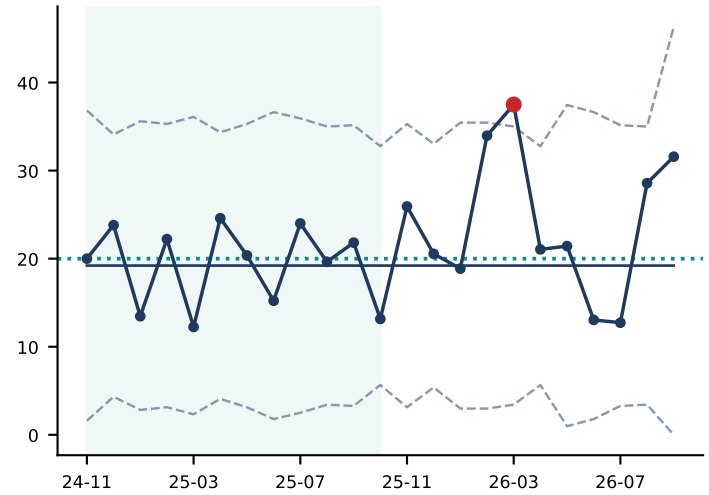
Safety events closed within 30 days (%)



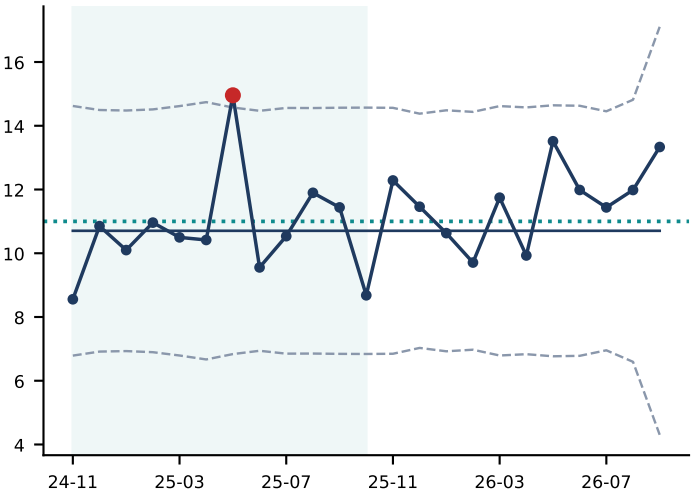
Medication events per 1,000 patient days



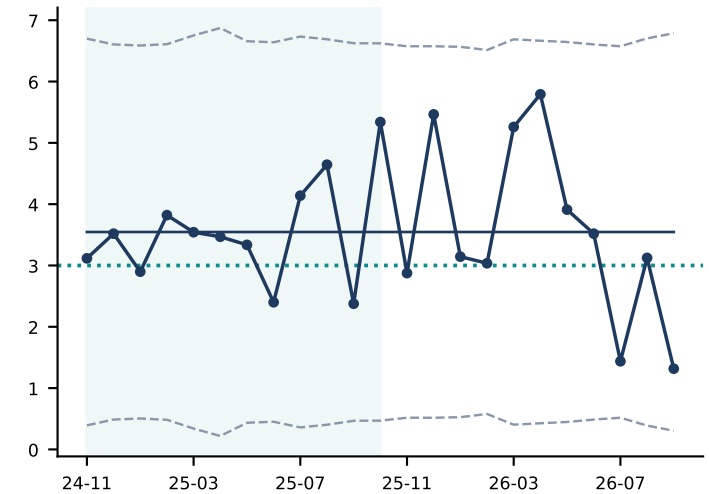
30-day readmission, heart failure (%)



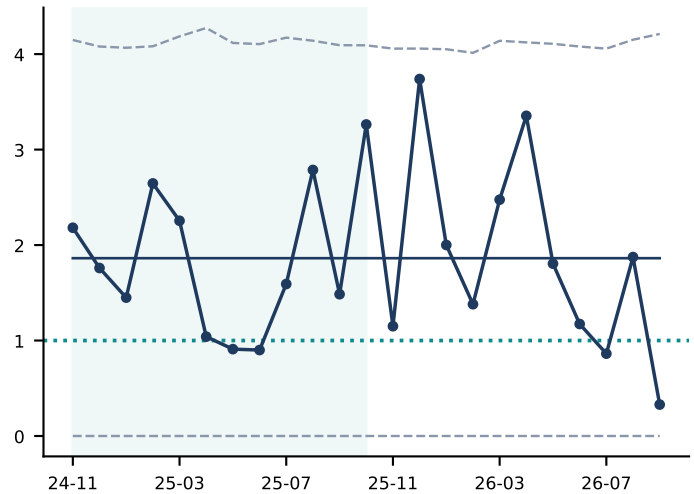
30-day all-cause readmission rate (%)



Falls per 1,000 patient days



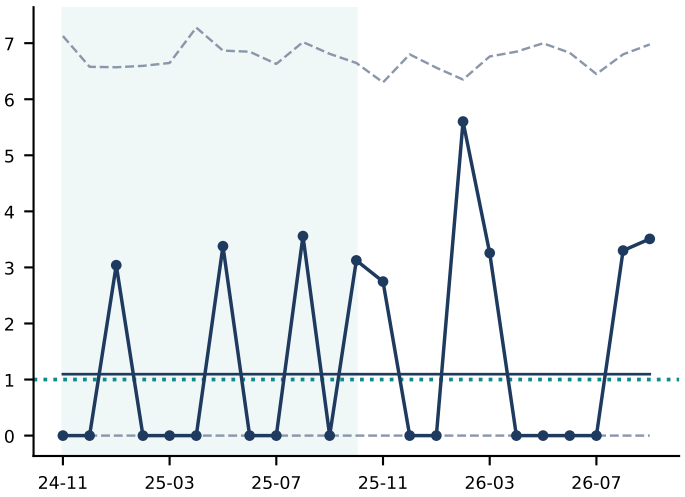
Falls with injury per 1,000 patient days



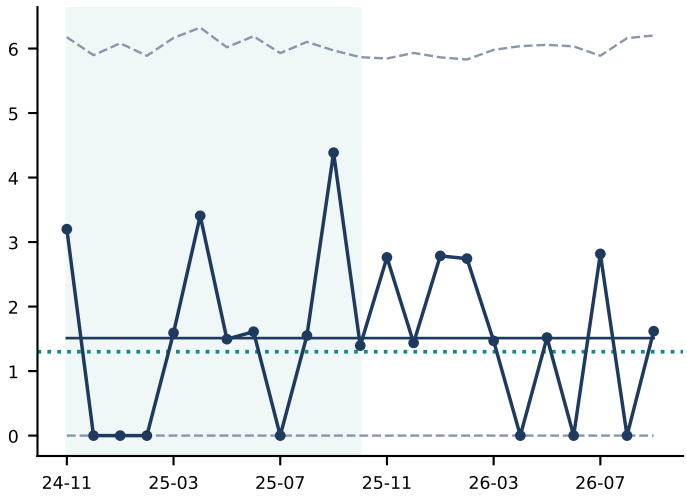
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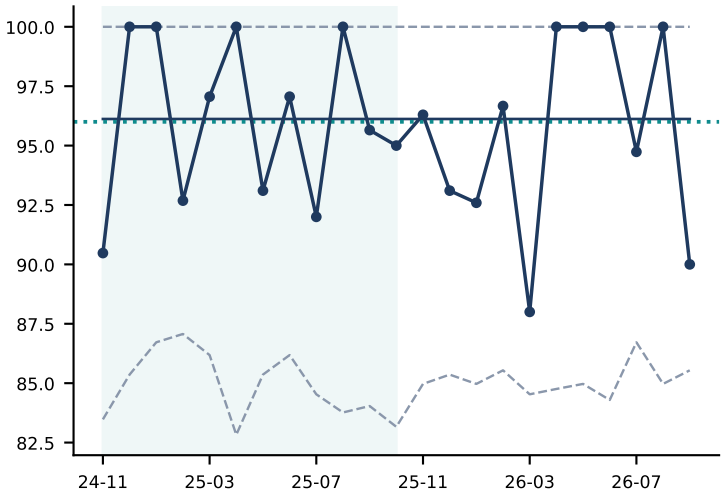
CLABSI per 1,000 central-line days



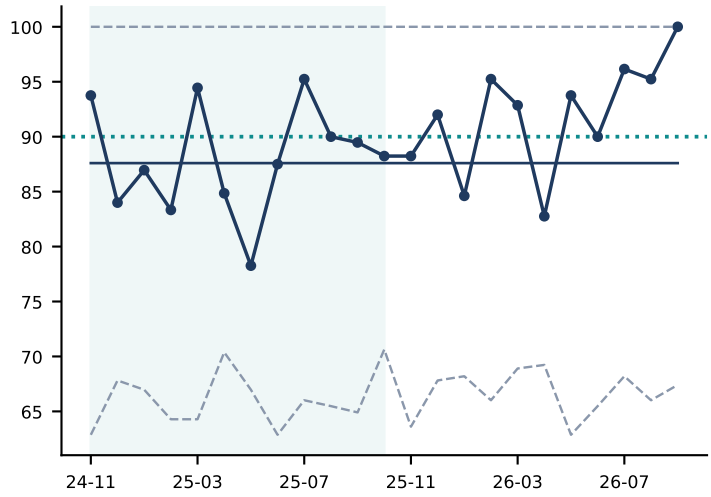
CAUTI per 1,000 catheter days



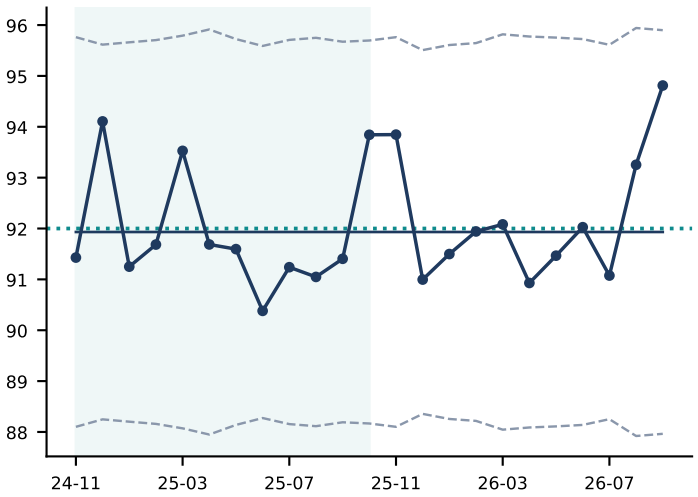
AMI aspirin within 24h of arrival (%)



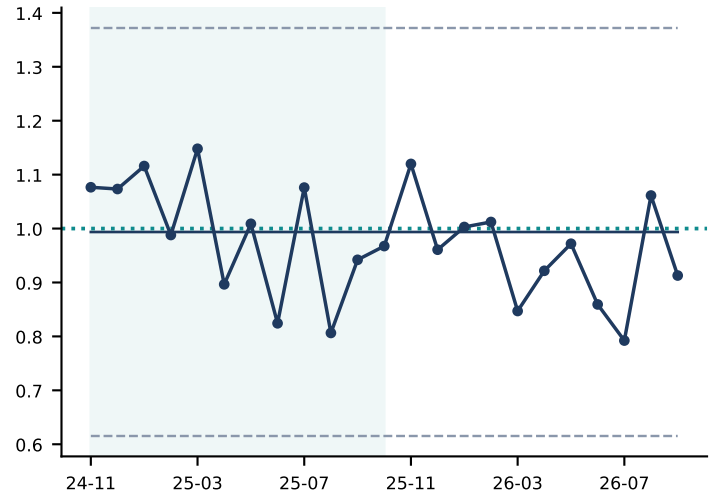
HF with LVSD: ACEI/ARB/ARNI at discharge (%)



VTE prophylaxis by end of admission day 1 (%)



Inpatient mortality, observed / expected



Abstraction & public reporting

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Abstractor workload (as of extract date)

abstractor	cases_total	completed	open_cases	overdue	avg_days_to_complete
A. Rivera	592	565	27	0	14.0
B. Okafor	610	586	24	0	15.1
C. Nguyen	514	487	27	3	14.0
D. Patel	293	284	9	0	14.2

Public-reporting submissions (most recent due)

Measure	Quarter	Due	Submitted	Status
AMI_ASA	2026Q1	2026-07-29	2026-07-16	Submitted on time
HF_ACEI	2026Q1	2026-07-29	2026-07-21	Submitted on time
SEP1	2026Q1	2026-07-29	2026-07-11	Submitted on time
VTE_PPX	2026Q1	2026-07-29	2026-07-12	Submitted on time
AMI_ASA	2025Q4	2026-04-30	2026-04-21	Submitted on time
HF_ACEI	2025Q4	2026-04-30	2026-04-23	Submitted on time
SEP1	2025Q4	2026-04-30	2026-05-03	Submitted late
VTE_PPX	2025Q4	2026-04-30	2026-04-23	Submitted on time
AMI_ASA	2025Q3	2026-01-28	2026-01-16	Submitted on time
HF_ACEI	2025Q3	2026-01-28	2026-01-20	Submitted on time